HEALTH ADMIN

The Appointment Companion

Prepare less frantically. Remember more clearly.

This is a paperwork companion, not a medical guide. It helps you walk in prepared and walk out with the next step written down.

INSIDE THIS GUIDE

01 A one-page appointment prep sheet
02 Questions that get clearer answers
03 After-visit and results trackers
04 Refill, referral, and insurance routines
05 Clear rules for AI health admin

PLAIN ENGLISH ON PURPOSE

Print it. Write on it.
Make it yours.

PLAIN-ENGLISH FIELD GUIDE · 10

Prepare without panicking

Appointments move fast. You wait weeks, then sit in a bright room and try to remember the one thing you needed to ask while someone types. Later, you are handed instructions you did not fully absorb. This planner exists to make that sequence less brittle.

> WATCH OUT
>
> **This is education, not medical advice**
>
> This companion helps with preparation and organisation. It does not diagnose, treat, or tell you to change medication. Follow the instructions from your own care team. If you think you are having an emergency, contact your local emergency number or emergency service now.

Before you start

- Use a pen and this page, or your phone's notes app.
- Have your appointment date, time, location, and clinician's name nearby.
- Keep your medication list and insurance card where you can find them.
- Decide whether you want someone with you for support.
- Do not paste private health records into a consumer AI tool while preparing.

What "prepared" means

It does not mean knowing everything. It means bringing the facts you have, naming what you need, and leaving with the next step in writing. That is enough.

Bring

Appointment details, medication list, insurance or payment information, questions, a pen, and a way to take notes.

Ask

What is happening? What are the options? What happens next? Who do I contact? What should I watch for? Write the answers down.

Leave with

The next appointment, any referral, test instructions, medication changes (if any), and a phone number for follow-up questions.

A note on notes

You can ask, "May I record this so I can remember it?" Some clinics allow it and some do not. Ask first. If recording is not allowed, ask the clinician to slow down or write the key points for you. Taking notes on paper is allowed and useful.

If you are supporting someone

Ask the person what they want help with before the appointment. They may want you to take notes, ask a specific question, drive, or simply be present. Being in the room is not permission to speak for them. Agree on a signal for "please step in" and one for "I've got this."

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The appointment prep sheet

Fill this in once, then bring it with you. Use short phrases. You do not need complete sentences.

The basics

APPOINTMENT	DATE AND TIME	CLINICIAN OR SERVICE	LOCATION / PHONE

Why I am going

Write the main reason in one sentence. Then list any changes since the last visit: symptoms, sleep, appetite, mood, pain, function, or side effects. Keep it descriptive; you do not need to diagnose it.

What I need to tell them

- ☐ A new or worsening problem.
- ☐ A medication question or side effect.
- ☐ A change in insurance, pharmacy, or address.
- ☐ A result or referral I do not understand.
- ☐ Something I have been avoiding mentioning.
- ☐ A practical barrier: transport, cost, time, caregiving, accessibility.

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Questions I want answered

Choose the three that matter most. You can add more later if there is time.

MY QUESTION	THE ANSWER (LEAVE BLANK UNTIL THE VISIT)
1.	
2.	
3.	

Getting there

TRANSPORT PLAN	TIME TO LEAVE	SUPPORT PERSON	WHAT I NEED TO BRING

TRY THIS

Put the appointment in two places

Add it to your calendar with the address and a travel alarm, then put the appointment card or note where you will see it. For an early appointment, set out clothes and paperwork the night before.

What I need to bring

- ☐ Appointment details and any paperwork they asked for.
- ☐ Medication list, if I take anything regularly.
- ☐ Insurance or payment information.
- ☐ My top three questions.
- ☐ A pen and paper, or a note-taking plan.
- ☐ Water, a snack, or anything that makes waiting easier.

One question I almost left out

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Ask until it is clear

You are allowed to ask questions. You are allowed to ask the same question in a different way. You are allowed to say that you did not understand. This is not being difficult; it is part of getting safe care.

Six questions worth keeping

01 **What do you think is going on?** Ask for the explanation in plain language.

02 **What are the options, including doing nothing or waiting?** Ask about the trade-offs.

03 **What is the next step, and who does it?** Make the action and owner concrete.

04 **How will we know whether it is working?** Ask about follow-up and warning signs.

05 **What should I do if the plan is not working or I get worse?** Write the answer down.

06 **Can you write the key points down for me?** This is a reasonable request.

The teach-back

Before you leave, say: “Can I repeat this back to make sure I have it?” Then explain the plan in your own words. This helps catch a misunderstanding while you are still in the room. You can also ask a support person to repeat it.

> GOOD TO KNOW
>
> **“I don’t understand” is a complete sentence**
>
> If an explanation is full of terms, ask: “What does that mean in everyday words?” or “Can you say that again more slowly?” A clinician would rather explain than have you leave confused.

Writing the answer

Do not rely on memory alone. Write the answer next to the question, even in messy shorthand. If you take notes in a phone, create one note per appointment. If someone comes with you, ask them to capture the practical details while you focus on the conversation.

When you need to interrupt

If you are overwhelmed, breathe and say: “I’m getting overwhelmed. Can we pause for a second?” If you need the room quieter, a break, a written summary, or a support person, ask. A short pause is better than leaving with information you cannot use.

My question for next time

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After the visit

The most useful ten minutes of an appointment can happen right after it ends. Sit somewhere quiet, before the details blur, and write four things: the plan, the next action, the follow-up, and who to call.

The ten-minute capture

- ☐ What is the main plan in one sentence?
- ☐ What changes, if any, were made to my care routine?
- ☐ What do I need to do next, and by when?
- ☐ What tests, referrals, or appointments were ordered?
- ☐ Who do I call with a question?
- ☐ What warning signs or instructions did they give me?
- ☐ What did I not understand?

My visit summary

WHAT WE DISCUSSED	WHAT HAPPENS NEXT	BY WHEN	WHO I CONTACT

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Follow-up and referrals

APPOINTMENT OR REFERRAL	ORDERED ON	SCHEDULING CONTACT	APPOINTMENT DATE	DONE
				☐
				☐
				☐

The next appointment

DATE	TIME	LOCATION	WHAT TO BRING	TRANSPORT PLAN

WATCH OUT

If instructions conflict or something feels wrong

Do not guess. Contact the care team that gave you the instructions, or use the after-hours number they provided. If you think it is an emergency, contact your local emergency number. This printable cannot tell you what is medically right.

My one unresolved question

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Tests, results, and the waiting gap

Waiting for a result can turn every day into a low hum of dread. This page does not make the result arrive faster. It gives the wait a container and makes it less likely that a follow-up falls through.

The result tracker

TEST OR SCAN	DATE DONE	WHERE RESULT WILL APPEAR	HOW I WILL BE TOLD	FOLLOW-UP DATE	DONE
					☐
					☐
					☐
					☐

The waiting plan

Choose what you will do while waiting. This is not about distraction at all costs; it is about giving the anxious part of your brain a known next checkpoint.

- ☐ I know when to expect the result.
- ☐ I know how the result will reach me.
- ☐ I know who to call if it does not arrive.
- ☐ I have a follow-up date in my calendar.
- ☐ I have one small thing to do on the day the result is due.
- ☐ I have someone I can check in with.

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When the result arrives

Do not try to interpret a result by yourself from a number on a screen. Write down what the result says, whether your care team has reviewed it, and what they told you to do next. If you do not understand a word or a range, ask. Online explanations may not apply to your situation.

> GOOD TO KNOW
>
> **A result is not a diagnosis by itself**
>
> Test results need context: why the test was ordered, your history, and what your care team is looking for. An AI chatbot does not have that context and cannot safely interpret your result for you.

If nobody calls

If the expected date passes, use the contact route your clinic gave you. A short message is enough: “I had [test] on [date]. I was told results would be available by [date]. Can you tell me whether they have been reviewed and what the next step is?” Record the response.

My result-day plan

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Refills, pharmacy, and the repeat stuff

Refills fail for boring reasons: the bottle ran out on a Sunday, the pharmacy needs a new prescription, the insurance changed, or the automatic refill was never set up. A small routine prevents the last-minute scramble.

> GOOD TO KNOW
>
> **Medication changes belong to your care team**
>
> This page helps you track and request refills. It does not advise starting, stopping, or changing a medication. If you are worried about a medication or a side effect, contact your clinician or pharmacist. Do not change how you take a prescribed medicine based on a chatbot's answer.

The refill routine

- ☐ Know the pharmacy name, address, phone, and hours.
- ☐ Know which medications need a new prescription before the pharmacy can refill.
- ☐ Check the label for refills remaining and the next eligible refill date.
- ☐ Put a refill reminder 5–7 days before you expect to run out.
- ☐ Request refills through the pharmacy's official app, phone line, or in person.
- ☐ Keep a current medication list with doses exactly as prescribed.
- ☐ Ask about refill synchronization if you have several prescriptions.

The medication list

Keep this page where you keep your appointment notes, and take it to appointments. Do not rely on memory if you take several medicines or have had recent changes.

MEDICATION (AS PRESCRIBED)	DOSE ON LABEL	HOW I TAKE IT	PRESCRIBER	PHARMACY	NOTES

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If the pharmacy says there is a problem

Write down exactly what they say before you panic. Common issues include:

- Refills remaining: 0 — a new prescription may be needed.
- Too soon to refill — check the date you are eligible.
- Insurance or prior authorization issue — ask what the next step is.
- Out of stock — ask when it will arrive or whether another pharmacy has it.
- Different formulation or dose — do not accept a change without confirming with the prescriber.

The travel and emergency plan

If you will be away from home, ask your pharmacy about an early refill or a vacation supply, and check the rules that apply to your medication. Keep medication in its labelled container. Do not transfer it into an unlabeled pill organizer for travel.

My refill reminder

My pharmacy details

PHARMACY	PHONE	HOURS	REFILL LINE OR APP

What I will ask when I call

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Insurance paper, medical bills, and the phone call

Health paperwork has its own vocabulary. You do not need to become an expert. You need to know what the paper is asking, whether it is a bill, and what the next step is.

Three documents that look alike

DOCUMENT	WHAT IT USUALLY MEANS	WHAT TO CHECK
Explanation of benefits (EOB)	A statement of what was billed, allowed, and paid. It is often not a bill.	Whether it says “You owe,” and whether the amount matches what you expected.
Provider bill	A request for payment from a clinic or hospital.	The date of service, the amount, and whether insurance has processed the claim.
Insurance denial or request for information	The plan needs more information or has declined coverage.	The reason, the deadline, the official appeal process, and who can help.

Before you pay a medical bill

- ☐ Confirm it is a bill, not an EOB.
- ☐ Check the patient name, date, and service.
- ☐ Compare it with the EOB if you have one.
- ☐ Ask for an itemised bill if the charges are unclear.
- ☐ Ask whether the claim has been processed by insurance.
- ☐ Ask about financial assistance, payment plans, or a prompt-pay discount.
- ☐ Write down the reference number and the date you called.

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A clear phone script

> “I received a statement for [service] on [date]. I want to confirm whether this is a bill and whether the insurance claim has been processed. Can you explain the amount and send me an itemised statement? I would also like to know about payment plans or financial assistance.”

Do not share payment information with someone who calls you unexpectedly. Hang up and use the number on your official statement or the insurer’s website.

GOOD TO KNOW

Appeals and coverage rules are specific

Appeal deadlines, coverage rules, and financial assistance programs vary. Ask the insurer, the provider, or a qualified patient advocate for the official process. This guide is education, not legal or benefits advice.

Before the call

- ☐ The statement or letter is in front of me.
- ☐ I know the date of service and the amount in question.
- ☐ I know whether I am asking about a bill, an EOB, or a denial.
- ☐ I have a pen and a place to write the reference number.
- ☐ I know the next step if this office cannot help.

The call record

DATE	WHO I SPOKE TO	REFERENCE	WHAT THEY SAID	NEXT STEP

My paperwork note

DOCUMENT	DATE	AMOUNT	WHAT IT IS	NEXT STEP

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AI can help with the admin, not the medicine

AI is useful for the organisational and language parts of health admin. It is not a clinician, does not know your history, and can invent details with confidence. Keep the line clear.

Fair uses

- Turn a list of dates into an appointment tracker.
- Suggest questions to ask your clinician.
- Rewrite a confusing insurance letter in plain language.
- Draft a polite scheduling or billing message.
- Help you make a checklist for what to bring.
- Practise saying “I need to understand the next step.”

Not safe uses

- Interpreting a test result or scan.
- Diagnosing a symptom.
- Starting, stopping, or changing medication.
- Deciding whether an emergency is “real enough.”
- Uploading private records to a consumer tool without understanding the settings.
- Letting AI write a message that claims something happened when it did not.

WATCH OUT

Do not paste what you would not want exposed

A health record can contain your name, date of birth, provider, diagnosis, and insurance details. Use placeholders. If you need help understanding a real document, ask the provider or insurer, or use a tool your care team has approved.

Prompt: prepare for an appointment

PROMPT RECIPE — HELP ME PREPARE, WITHOUT DIAGNOSING

I have an appointment about [general concern]. Ask me up to five questions to help organise what I want to say. Then give me a short prep sheet with: my main concern, changes to mention, three questions for the clinician, and what I should write down during the visit. Do not suggest a diagnosis or a treatment. Remind me to confirm every next step with my care team.

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Prompt: understand an official letter

PROMPT RECIPE — MAKE THIS ADMIN LETTER EASIER TO ACT ON

Rewrite the following generic sentence from an insurance or clinic letter in plain English: [paste a sentence with all names, IDs, and dates removed]. List the action it seems to require and three questions I should ask the sender. Do not guess the policy or tell me what the letter legally means.

Prompt: draft a scheduling message

PROMPT RECIPE — DRAFT A CLEAR SCHEDULING MESSAGE

Write a short message to [clinic or service] asking about [appointment, referral, or bill]. Include my name placeholder, the date of service if relevant, and a request for the next step. Keep it polite and under 120 words. Do not invent any medical details or claim that I have already done something.

My AI boundary

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A small health-admin kit

You do not need a complete medical binder. You need a place where the next appointment, the current medication list, the latest result, and the insurance question can live together. Make the system smaller than the chaos.

The folder

Create one physical folder or one secure digital folder called “Health admin — current.” Inside, keep:

- Appointment cards and recent visit summaries.
- The current medication list.
- Results and letters you are waiting on.
- Insurance and billing papers that need action.
- A single page of current questions.

Review it before an appointment and after one. Once a quarter, remove papers you no longer need and check that the medication list is current.

The monthly five-minute check

- ☐ Any appointment in the next two weeks?
- ☐ Any test result or referral waiting?
- ☐ Any medication or refill due?
- ☐ Any bill or insurance letter unresolved?
- ☐ Any question I want to remember to ask?

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My health-admin home

Physical folder location:

Secure digital folder or password manager note name (never the password):

Person who knows where it is, if that is useful:

TRY THIS

One question is enough

If you only bring one question to an appointment, bring the one that would change what you do next. Write it at the top of the page. The rest can wait.

My next small step

GOOD TO KNOW

A final reminder

Preparation is not a substitute for care. It is a way to make the care you receive easier to understand, follow, and remember.

ONE LAST THING · 10

Use one thing. Then keep it.

Pick your next appointment and fill in only the top three boxes: why you are going, your top question, and how you will get there. The rest can wait.

MY NEXT SMALL STEP

THE RULE FOR THIS HALF OF THE PACK

A system should fit your real energy, not the other way around. Keep what helps, drop what becomes another chore, and let the next small step be enough.

More life-admin printables

07 The Low-Energy Weekly Reset

08 The Paper Pile Triage Kit

09 Bills, Renewals & Subscriptions

11 The Low-Energy Meal Plan

12 The Household Command Center

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Keep the useful part. Leave the rest.